Hello, Mr Alvarez. Your GP has sent us the letter about your problems, but why don't you tell me in your own words what's been happening.

M *Ah, yes, well, I've had blood in my urine and like this sharp kind of **(1) chest pain**. I had an ECG for that but it didn't show anything serious, but the pain was still there. So, my doctor sent me for an ultrasound of my **(2) abdomen**, but the result was all right. I think that's why he's sent me to see you.*

F *Yes, and you had a normal **(3) endoscopy** two years ago. Well, how are you now?*

M *My stomach always **(4) feels full up**. It's just the same as before. It's been happening for at least 18 months now. If I'm lucky, on a good day I'll have no pain at all, but on a bad day it's pretty constant.*

F *Does it ever wake you up at night?*

M *I sleep very well. I usually wake up because of **(5) poor breathing**.*

F *Ever had any problems swallowing food or liquid?*

M *No, nothing like that. But it can feel like it's burning on the way down if I drink something hot.*

F *I see. Anything else you can think of?*

6Acidic

M *I often burp a lot after eating or drinking. That actually helps me feel better. I've never really* ***(7) vomited****, even when I've been feeling my worst, but I can get this terrible taste in my mouth when I"m****(8) burping****. It's ah... it's like something very bitter, sour almost.*

F *Have your ever coughed up any blood or noticed any blood in your stools?*

M *No, thank goodness.*

F *Okay. Are you on any medication?*

M *I used to take tablets for my ulcer…ah…they were red and yellow…* ***(9) Losec****, that's them! I'd take one for every twenty days. They seemed to work quite well as I remember. That's it though. I don't really like to take a lot of pills.*

F *It sounds like apart from this trouble, you're quite well then.*

M *Yes, touch wood. My wife and I eat a pretty healthy diet, I think. I never go out to pubs, but I have a glass of wine with dinner a few nights a week. I used to smoke (****10) 10 to 15 cigarettes*** *a day, but I quit two months ago.*

F *What about your family's medical history?*

M *Well, I ah, I have seven brothers and two sisters. They're all well and living in Spain. My mother died at ninety of a heart attack and my father at seventy-seven of a* ***(11) stroke****. Like I said, I've never really had any problems myself apart from this pain. I suppose I'm worried in case it's* ***(12) cancer****.*

F *Well, all of your tests have been fine, and your GP did a heart rate test and that was all right too. I'd like to examine you, and send you for some blood tests... (*FADE OUT)

Your GP says you've been to see him about a lump in your breast. So, you when did you first notice this lump?

F ***(1) Ten days ago**. I was doing my routine check when I was just so shocked to find it. It's in my **(2) left breast**. I'm really worried because my **(3) mum died** six months ago from breast cancer and I've been scared of getting it ever since*

M *How old was your mum?*

F *Sixty-five.*

M *Do you know if anyone else in your family has had it?*

F *I don't believe anyone else has. I don't remember either of my parents talking about it affecting their own mums.*

M *Are you married?*

F *Yes, but we **(4) separated** nine months ago. He went off with another woman and I'm on my own looking after my two children. I've got a girl who's 14 and a 12-year-old boy as well.*

M *Have you ever been sick with anything serious before?*

F *No, my health's always been very good.*

M *I take it you're not on any **(5) medications** then.*

F *That's right, nothing. But there is something else that's been playing on my mind since I noticed the lump. I started remembering this pain I've been getting just here, in my chest. I never really thought anything of it before because it was kind of…oh, I don't know, just very dull, like I'd* ***(6) pulled****something, a* ***muscle*** *of some sort. So I really wasn't very worried about it. I just thought it would go away in time. But now, I don't know. Do you think it could be a sign of something?*

M *That's difficult for me to say until I examine you and we run some tests. Is there anything else you can take me?*

F *About the pain? Well, it seems to go to my* ***(7) back****. I usually only notice it when I'm sitting in a chair, not a chair at the kitchen table, more like when I'm sitting in the lounge. You know, something with a high back on it. It did* ***(8) wake*** *me from sleep once. It only lasted for about 15 to 20 minutes, but it was probably the worst I'd felt it. There hasn't been anything else really. Like I said before, I'm usually pretty healthy.*

M *You haven't had any nauseas or vomiting with it? No shortness of breath?*

F *Nothing else except for what I mentioned. Oh, I do hope everything's all right.*

M *Of course. Do you mind I=if I examine you now so I have an ideas of what we're dealing with?*

F *No, that's fine.*

M *Okay, there it is. Does that hurt?*

F *No.*

M *Right, it's quite* ***(9) solid****. Roughly* ***(10) 2 or 3 centimetres*** *in length, I'd suggest. We'll need to run some tests to confirm exactly what it is. We can do those today, now actually, and then wait a few days for the results to come in. I'll need you to make a follow-up appointment on your way out today.*

F *Okay, that sounds fine. I know I'm probably getting ahead of myself here, but if it is cancer, I really don't want to lose the breast. I'd hate to have to have a* ***(11) mastectomy****. I dread that probably more than anything else. I really don't care what else happens so long as I don't lose the breast. My mother had* ***(12) chemotherapy*** *after she was diagnosed with cancer, and even*

though she lost all her hair, I'd be willing to try it at least if the hair grew back.

M *Okay, well, you are jumping in a but hearly here, but there may be options. First let's run the tests and wait for the results.. (Fade out)*

You hear a junior doctor speaking with a patient in the cardiac clinic while a consultant is present.

What does the patient suggest about their pain?

Select one:

- A. it affects his breathing.
- B. it is located mainly in his chest.
- C. it makes it difficult for him to go walking.

Feedback

Your answer is correct.

Transcript

F *Hello again, Mr Waites. We met last week, didn't we? I've been telling my colleague here about you. I've seen the results of the tests but there are just one or two points I'd like you to clarify for me and for the benefit of Dr Granger. You did say that this pain and discomfort has troubled you for nearly eight months and is getting worse?*

M *That's right, doctor.*

F *Can I get you to describe the character of the pain - or in other words, tell us what it is like exactly?*

M *It's not always a pain as such. Often it's a feeling of discomfort in my chest, like someone's pressing down on it.*

F *What about your breathing when you get the pain?*

M *I don't notice any difference.*

F *Now what would happen if you and I and Dr Granger here went for a walk up the hill to the station?*

M *What's that now?*

F *I just mean if we were to walk what do you think would happen? Do you think you could keep up with me if I walked fast?*

M *Oh yes. Well last summer I climbed up Ben Nevis in Scotland, so I've no problems there.*

The correct answer is: it is located mainly in his chest.

Information

Flag question

Information text

Question 2 Audio

Question 2
Correct
Mark 1.00 out of 1.00

Flag question

Question text

You hear a senior nurse talking about removing sutures to a group of students.

What is she doing?

Select one:

- A. Recommending a clear procedure to follow.
- B. Explaining why a clean environment is important.
- C. Suggesting that sometimes there may be complications.

Feedback

Your answer is correct.

Transcript

F *When removing sutures, put on an apron. If you have long hair, be sure that it is tied back securely. First, explain to the patient that you are going to remove their sutures (the patient will probably use the word stitches). Second, ask the patient if he or she is comfortable. The patient pay ask if it will hurt. Reassure the patient that removing sutures doesn't hurt. Third, wash and dry your hands. Then, prepare some gauze, cleansing solution and a stitch cutter on a sterile tray. Then, put your hand inside a sterile waste bag and use it like a glove to remove the bandage from the sutures. Turn the sterile bag inside out so that the bandage is now inside the bag. Put on sterile gloves. Finally, look at each suture and check for inflammation around them.*

The correct answer is: Recommending a clear procedure to follow.

Information

Flag question

Information text

Question 3 Audio

Question 3
Incorrect
Mark 0.00 out of 1.00

Flag question

Question text

You hear a staff nurse talking to a patient she is escorting to the Radiology Department.

What does she need to know from the patient?

Select one:

- A. If he is ready.
- B. How he is feeling.
- C. His personal details.

Feedback

Your answer is incorrect.

Transcript

F *Hi Jake, it's time for your X-ray now. Are you ready?*
M *Oh, okay.*
F *Can you walk to the Radiology Department, do you think?*
M *Is there a wheelchair or a walking stick, maybe? I'm a bit weak this morning.*
F *There's a wheelchair.*
M *That'd be great, thanks.*
F *Of course. Let me help you. There you are. Are you warm enough?*
M *I'm a bit cold actually.*
F *Let me get you a blanket. There. Is that better?*
M *Thanks.*
F *And I just need to see your identity bracelet, please. Thanks. What's your full name?*
M *Jake Peterson.*
F *Your date of birth?*

M *18th of January, 1982.*

F *Okay, thanks.*

The correct answer is: His personal details.

Information

Flag question

Information text

Question 4 Audio

Question 4

Incorrect

Mark 0.00 out of 1.00

Flag question

Question text

You hear a nurse talking with a patient in the recovery unit.

What is the patient worried about?

Select one:

A. Her breathing difficulties.

B. The pain in her chest.

C. How cold she feels.

Feedback

Your answer is incorrect.

Transcript

M *Mrs Wendell, can you hear me?*

F *Yes.*

M *I'm your recovery room nurse. Your heart bypass surgery went well.*

F *Good.*

M *I'm going to take your vital signs every 15 minutes and check your oxygen levels. This mask will help with. Can you breathe well?*

F *Yes. But my chest hurts.*

M *On a scale of one to ten, one being no pain at all, ten being the worst pain, what number is your pain right now?*

F *I guess 4 or 5.*
M *Okay. Do you feel nauseous?*
F *No, but I feel cold.*

The correct answer is: The pain in her chest.

Information

Flag question

Information text

Question 5 Audio

Question 5
Correct
Mark 1.00 out of 1.00

Flag question

Question text

You hear the beginning of a training session for doctors.

The trainer explains that the session will focus on

Select one:

- A. how hormones work.
- B. the purpose of various glands.
- C. problems with the endocrine system.

Feedback

Your answer is correct.

Transcript

F *The function of the endocrine system is to regulate the various organs within the body and it does this by using hormones, which are produced by glands. Today we're going to look at ways in which the endocrine system can malfunction and the effects that that can produce. I would like to begin by looking at the functioning of the thyroid gland. This is one of the largest glands in the body and it is located in the neck, below the mouth. It produces a number of hormones, the most important of which are T3 and T4 … (FADE OUT)*

The correct answer is: problems with the endocrine system.

Information

Flag question

Information text

Question 6 Audio

Question 6
Correct
Mark 1.00 out of 1.00

Flag question

Question text

You hear a doctor talking to a nurse about a patient called Mr Kransky.

The doctor wants the nurse to

Select one:

A. ensure the patient can feed themselves by the end of the week.

B. contact the OT about the patient's swallowing reflex.

C. continue the patient on their current diet.

Feedback

Your answer is correct.

Transcript

M *Can you tell me how Mr Kransky's doing?*
F *Mr Kransky? Mmm. He's doing quite well but he still has difficulty swallowing.*
M *Has he seen the speech and language therapist yet?*
F *Yes, he sees her every day to do tongue exercises to help his swallow reflex.*
M *Ihat's good. What type of diet is he on now?*
F *He started a pureed diet yesterday.*
M *How's he managing it?*
F *He's managing it quite well.*
M *What about fluids?*
F *He's still having thickened fluids and he's coping quite well with them.*
M *Okay. Ah... Can he feed himself yet?*
F *No, not yet. He still needs help.*

M *Okay. I'll write a referral to the OT.*

F *Thanks. Maybe she can bring a few things to help him feed himself.*

M *Right. Can you keep him on with the pureed diet and thickened fluids until his swallow reflex is better?*

F *Sure.*

M *I'll review him at the end of the week.*

The correct answer is: continue the patient on their current diet.

Roger believes radiation therapists are necessary because

Select one:

- A. there are so many different types of cancer.
- B. patients need help when battling cancer.
- C. it is an essential way to treat cancer.

Feedback

Your answer is correct.

Transcript

F *We're talking today with radiation therapist Roger Atkins, program director of Radiation Therapy at Hull University. So, Mr Atkins, why do we need radiation therapists?*

M *Cancer encompasses over 125 diseases. In the UK alone, over 1 million people will be diagnosed with cancer this year. Approximately 70 percent of the cancer patients will receive radiation therapy. The goal of radiation therapy is to eradicate cancer cells, while at the same time sparing normal cells. Because radiation therapy is such an important part of treating cancer, there is a growing need for competent, caring radiation therapists in all parts of the world. As a medical radiation professional it can be confronting to experience loved ones going through cancer and radiation therapy. But it is also very important the way that we communicate and deliver education to patients throughout their cancer journey.*

The correct answer is: it is an essential way to treat cancer.

Question 2
Correct
Mark 1.00 out of 1.00

Flag question

Question text

What led Roger into becoming a radiation therapist?

Select one:

- A. He was inspired by the man who treated his mum.
- B. He wanted to save people after his mum died from cancer.
- C. He learnt so much while his mum was dying and wanted to put it to use.

Feedback

Your answer is correct.

Transcript

F *What inspired you to become a radiation therapist?*

M *My career as a radiation therapist is directly linked to my own mother dying of ovarian cancer during my junior year of college. She was diagnosed with cancer when I was a college student. As I accompanied her during cancer treatments over the next two years, I learned about the various people who worked with cancer patients. I remember the radiation therapist who treated my mum. He was a very compassionate man, a good communicator. He was always happy to listen to her, to her concerns and anxieties. Listening to him, the way he worked with my mum, it had a lasting impact on me. Although I was originally a history major, I came to the realisation that my life's work would be working with and caring for cancer patients. From this personal experience, I chose a career in radiation therapy.*

The correct answer is: He was inspired by the man who treated his mum.

Question 3
Correct
Mark 1.00 out of 1.00

Flag question

Question text

Roger says that when treating patients, radiation therapists

Select one:

- A. often work very long hours.
- B. regularly use a range of approaches.
- C. need to be aware of dangerous emission levels.

Feedback

Your answer is correct.

Transcript

F *What does a radiation therapist do?*
A radiation therapist is a valuable member of a cancer treatment team who delivers radiation treatments to patients, helps construct treatment plans, and works with cancer patients and their families. Cognitive, psychomotor, and affective thinking skills are used every day. When a patient is treated with radiation therapy, they can only receive a limited amount of radiation dose each day. Consequently, it takes anywhere from two to eight weeks
M *for a patient to complete their treatment regimen. Because they usually see the one patient five days per week for so many weeks, radiation therapists often develop close relationships with their patients. The radiation therapist also needs to be able to plan, evaluate, and assess several parameters for each patient every day. The radiation therapist will also assess the patient's reaction to treatment, providing advice on the side effects of treatment and methods of alleviating these.*

The correct answer is: regularly use a range of approaches.

Question 4
Correct
Mark 1.00 out of 1.00

Flag question

Question text

Roger mentions the case of his former patient, Susan, to

Select one:

- A. show that not everyone feels the same about radiation therapy.
- B. highlight the types of equipment used in radiation therapy.
- C. explain the steps involved in radiation therapy.

Feedback

Your answer is correct.

Transcript

F *Can you describe an individual case to us?*

Well, I recently treated a patient called Susan. Now Susan was quite comfortable in the radiation therapy setting because she herself had previously worked in a radiology practice. She even went as far as to tell me that she felt cared for in that environment. Her husband,

M *however, had a different reaction to Susan's radiation therapy altogether. He found the linear accelerator confronting, he had trouble reconciling radiation dose as a cancer-killing mechanism (after all, doesn't radiation also kill you?) and thought the advances of science would mean that Susan would not be treated on an imposing machine in a concrete bunker.*

The correct answer is: show that not everyone feels the same about radiation therapy.

Question 5

Correct

Mark 1.00 out of 1.00

Flag question

Question text

Why does Roger find teaching radiation therapy rewarding?

Select one:

- A. He likes knowing his students are making a difference.
- B. There is a lot that that he has learnt that he can pass on.
- C. It provides opportunities for him to work with past patients.

Feedback

Your answer is correct.

Transcript

F *What would you say is the most rewarding aspect of your career?*

First, seeing and speaking with patients who I treated 10–15 years ago and having them give me a hug is very rewarding. Second, as a radiation therapy teacher I see the accomplishments of our program's alumni and the many lives they have touched, cared for,

M *and helped cure. We are very proud of each graduate. Their work is noble and greatly benefits humanity. When I teach, I don't only talk about radiation therapy, but also discuss how the use of radiation can benefit mankind and improve the quality of our lives through both medical imaging and radiation therapy.*

The correct answer is: He likes knowing his students are making a difference.

Question 6

Correct

Mark 1.00 out of 1.00

Flag question

Question text

What does Roger suggest about cancer treatment in the future?

Select one:

a. Further research is needed to reduce instances of the disease.

b. More patients should be encouraged to talk about their condition.

c. Physicians need to do a better job of reducing patient stress.

Feedback

Your answer is correct.

Transcript

F *And finally, how do you see the future for radiation therapy and cancer treatment?*

M *Well, I think quite a lot about the importance of research focusing on patients' perspectives and improving the way that we communicate and deliver education to patients throughout their cancer journey. It may well be that patients are nervous or anxious when they present to radiation therapy because they have had a difficult experience in another health setting or perhaps have already experienced chemotherapy side effects. Then there's the issue of patients who have a family and young children. Patients in this situation face a quandary in deciding what to tell their children, and each patient manages this experience differently. Many parents want to protect their children from distress and so do not communicate with children about their illness, or they give them limited information. However, clear and open communication in an age appropriate way emerges strongly across a number of studies, and quality communication between parents and children is associated with emotional health when a parent has cancer.*

The correct answer is: More patients should be encouraged to talk about their condition.

Dr Werner suggests that when it comes to vitamin D production

Select one:

A. the sun is important.

B. too much sun can be dangerous.

C. without the sun we increase our chance of illness.

Feedback

Your answer is correct.

Transcript

F *Hello. My name is Dr Anne Werner. I'm a dermatologist and author. Today I'm going to talk about vitamin D production. We all need vitamin D. It spurs bone growth, and without it we'd be at high risk of conditions such as muscle aches, muscle weakness, bone pain, osteoporosis and fractures. One way we produce vitamin D is through our skin, which our skin manufactures when it is exposed to sunlight. The sun's ultraviolet B rays interact with a protein called 7-DHC in the skin, converting it into vitamin D3, the active form of vitamin D. The problem is that currently too many people think that using sunscreen and other forms of sun protection leads to vitamin D deficiency, and that the best way to obtain enough of the vitamin is through unprotected sun exposure. But that can lead to a whole other set of serious problems.*

The correct answer is: too much sun can be dangerous.

Question 2

Correct

Mark 1.00 out of 1.00

Flag question

Question text

What does Dr Werner say about the benefits of vitamin D?

Select one:

A. It eliminates a lot of serious disease.

B. More health practitioners should be supporting it.

C. There are a lot of unsubstantiated theories about it.

Feedback

Your answer is correct.

Transcript

F *Vitamin D helps keep your bones strong by regulating calcium levels. In recent years, some proponents of vitamin D have hypothesised that it does everything from decreasing cancer deaths and heart disease to inhibiting type 1 diabetes and rheumatoid arthritis. However, these suggestions are based on observational studies alone. This means researchers have observed that people with enough vitamin D have a lower incidence of deaths from these diseases, but not that vitamin D is the reason they are lower. While observational studies may be a good starting point, they are not proof or a basis for medical recommendations.*

The correct answer is: There are a lot of unsubstantiated theories about it.

Question 3
Incorrect
Mark 0.00 out of 1.00

Flag question

Question text

Dr Werner mentions the benefits of sun protection in order to

Select one:

- A. highlight the importance of covering up.
- B. provide statistics on the incidence of cancer.
- C. discuss the different types of energy produced.

Feedback

Your answer is incorrect.

Transcript

F *In contrast, there is overwhelming evidence for the multiple benefits for sun protection. Controlled studies have shown that regular use of an APF 15 or higher broad-spectrum sunscreen reduces your chances of developing squamous cell carcinoma by about 40 percent, melanoma by 50 percent and premature skin aging by 24 percent. It has been proven on the molecular level that the sun's ultraviolet (UV) light damages the skin's cellular DNA, creating genetic mutations that can lead to skin cancer. In addition, UV radiation harms the eyes and can cause cataracts, eyelid cancers and other ocular skin cancers. In short, unprotected sun exposure puts you at risk for any number of conditions that can permanently damage your skin, disfigure you, and sometimes even kills you. And the regular use of sun protection can go a long way to keep any of that from happening.*

The correct answer is: highlight the importance of covering up.

Question 4
Correct
Mark 1.00 out of 1.00

Flag question

Question text

Dr Werner believes using sun protection doesn't lead to vitamin D deficiency because

Select one:

A. most people use low-SPF sunscreens.

B. it is impossible to block all ultraviolet light.

C. radiation levels are higher than they used to be.

Feedback

Your answer is correct.

Transcript

F *High-SPF sunscreens are designed to filter out most of the sun's UVB radiation, since UVB damage is the major cause of sunburn and can lead to skin cancers. UVB wavelengths also happen to be the specific wavelengths that trigger vitamin D production in the skin. Nonetheless, clinical studies have never found that everyday sunscreen use leads to vitamin D insufficiency. In fact, the prevailing studies show that people who use sunscreen daily can maintain their vitamin D levels. One of the explanations for this may be that no matter how much sunscreen you use or how high the SPF, some of the sun's UV rays reach your skin. An SPF 15 sunscreen filters out 93 percent of UVB rays, SPF30 keeps out 97 percent, and SPF 50 filters out 98 percent. This leaves anywhere from 2 to 7 percent of solar UVB reaching your skin, even with high SPF-sunscreens. And that's if you use them perfectly.*

The correct answer is: it is impossible to block all ultraviolet light.

Question 5

Correct

Mark 1.00 out of 1.00

Flag question

Question text

According to Dr Werner, we don't need very much sun exposure because

Select one:

A. vitamin D deficiency is very rare.

B. too much vitamin D can cause problems.

C. even small quantities provide enough vitamin D.

Feedback

Your answer is correct.

Transcript

Even committed proponents of unprotected sun exposure recommend no more than 10 to 15 minutes of exposure to arms, legs, abdomen and back, two or three times a week, followed by good sun protection. That minor amount of exposure produces all the vitamin D your body can muster. After that, your body automatically starts to dispose of vitamin D to avoid an overload **F** *of the vitamin, at which point your sun exposure is giving you nothing but sun damage without any of the presumed benefit. The thing is, even just those unprotected 10 or 15 minutes are way more than enough time to cause DNA damage, and every bit of this damage adds up throughout your lifetime, producing more and more genetic mutations that keep increasing your lifetime risk of skin cancer.*

The correct answer is: even small quantities provide enough vitamin D.

Question 6
Correct
Mark 1.00 out of 1.00

Flag question

Question text

Dr Werner believes everyone can acquire the necessary allowance of vitamin D from

Select one:

A. a combination of food and supplements.

B. dietary supplements alone.

C. certain types of food.

Feedback

Your answer is correct.

Transcript

The question is then, if not from UV exposure, where should you obtain most of your vitamin D? Fatty fish such as salmon, mackerel and tuna are especially good sources. Small amounts are also present in egg yolks, beef liver and cheese. And many common foods such as milk and **F** *orange juice are fortified with vitamin D. It is possible then, though sometimes difficult, to mix and match these foods to get the daily allowance of 600 International Units (IU) recommended for the average person between the ages 1 and 70. However, if food sources aren't enough, you may need to add in a supplement. Remember though that most nutritionists believe food should*

always be your first choice, with supplements used as reinforcements. The bottom line is food, supplements and incidental, protected sun exposure will give you all the vitamin D you need.

The correct answer is: a combination of food and supplements.